

Scientist Network for Advancing Policy (SNAP)

snapcoalition.org/
Policy Hackathon 2026

Policy <> Track B Health Policy Innovation

About This Track:

This challenge is part of the Policy Hackathon 2026 B track: Health. Turn a real health-systems problem into a proposal that could move through a legislature or CMS.

Use Case Description:

This challenge asks teams to translate a real health systems problem — access, cost, equity, or workforce — into a policy proposal that could realistically move through a state legislature, Congress, or a federal agency such as CMS.

Problem Statement:

Translating a health-systems problem into a proposal that could realistically move through government.

- Choose a focus area: rural health access, behavioral health integration, aging/caregiving policy, maternal health, health data interoperability, or drug pricing.
- Ground the problem in data (prevalence, cost, disparities by geography/demographics).
- Review existing policy levers (state waivers, CMS rules, legislation, payer models).
- Design an intervention with an implementation pathway and funding mechanism.
- Identify equity impacts and unintended consequences.

Possible Approaches:

- State-level pilot — propose a Medicaid waiver or state demonstration program.
- Federal rule/legislation — draft a CMS rule comment or bill concept with committee jurisdiction identified.
- Payment-model — design a value-based care or reimbursement structure.
- Community-based — center on a community health worker / social determinants intervention.

Challenge Duration:

- Teams start working in September 2026
- Submission on **November 14, 2026**. There will be no extension to the submission deadline.

Team Guidelines:

- Team size — **3–5 participants per team, interdisciplinary encouraged.**
- Ensure you are enrolled as a participant in Policy Hackathon 2026.
- Everyone is eligible to participate in this challenge and win awards.
- Standout teams may be invited to brief partner organizations or judges directly on their proposal.

Challenge Tasks / Deliverables:

- 4–6 page policy brief with problem statement, evidence base, and recommendation

- Implementation roadmap (who does what, in what sequence)
- Budget/fiscal note
- 5-slide leadership briefing deck
- Process appendix (0.5 page)

Optional Advanced Tasks:

- Patient/provider journey map showing before/after the policy change
- Interview 1–2 practitioners or advocates (with consent)
- Draft sample legislative or waiver language
- Equity impact assessment using a recognized framework

Platforms and Tools:

- Data: CDC WONDER, KFF, CMS data.cms.gov, county health rankings
- Legislative tracking: Congress.gov, state legislature trackers, Regulations.gov
- Drafting/collaboration: Google Docs, shared repo

Judging Criteria:

- Evidence quality & data use — **25%**
- Feasibility (political + fiscal) — **20%**
- Equity analysis — **20%**
- Clarity of implementation plan — **20%**
- Presentation quality — **15%**

Data Privacy / Eligibility Restriction:

- No individually identifiable patient data (PHI) — aggregate/public data only.
- Informed consent required for interviews; anonymize unless attribution is agreed.
- Treat any shared personal health information as sensitive by default.
- Disclose AI tool use in an appendix.

Participants retain ownership of their submissions but grant Scientist Network for Advancing Policy (SNAP) the right to evaluate, discuss, and showcase submitted work for educational and research purposes, with appropriate attribution.